

Nausea and Vomiting During Early Pregnancy

(Morning Sickness)

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Pregnancy frequently causes nausea and vomiting. It is often called "morning sickness," but it can occur any time of day or night. This type of nausea and vomiting usually starts around 5 weeks of pregnancy and is at its worst at about 9 weeks. It usually disappears by about 16 to 18 weeks. Symptoms vary from mild to severe.

Hyperemesis gravidarum is a severe, persistent form of pregnancy-related vomiting. Women with hyperemesis gravidarum vomit so much that they lose weight and become dehydrated. Such women may not consume enough food to provide their body with energy. Then the body breaks down fats, resulting in a buildup of waste products (ketones) called ketosis. Ketosis can cause fatigue, bad breath, dizziness, and other symptoms. Women with hyperemesis gravidarum often become so dehydrated that the **balance of electrolytes**, needed to keep the body functioning normally, is upset. If women vomit occasionally but gain weight and are not dehydrated, they do not have hyperemesis gravidarum.

Causes of Nausea and Vomiting During Pregnancy

Usually, nausea and vomiting during pregnancy are related to the pregnancy. However, sometimes they result from a disorder unrelated to the pregnancy.

Pregnancy-related causes

The most common causes of nausea and vomiting related to pregnancy are

- Morning sickness (most common)
- **Hyperemesis gravidarum**

The causes of morning sickness and hyperemesis gravidarum are unclear. It is possible that these symptoms are related to a rapid increase in the levels of two hormones in early pregnancy: human chorionic gonadotropin (hCG), which is produced by the placenta early in pregnancy, and estrogen, which helps maintain the pregnancy. Also, hormones such as progesterone may slow the movement of the digestive system, possibly contributing to nausea and vomiting.

Prenatal vitamins with iron may cause nausea. [Gastroesophageal reflux](#), often with heartburn, is common in pregnancy.

Occasionally, the corpus luteum, an ovarian cyst that is normal in early pregnancy, may cause the ovary to twist around the ligaments and tissues that support it, cutting off the ovary's blood supply. This disorder, called [adnexal torsion](#), is not related to the pregnancy but is more common during pregnancy.

Rarely, severe, persistent vomiting results from a [molar pregnancy](#) (abnormal placental growth with or without a fetus due to an abnormally fertilized egg).

Other causes

Causes of nausea and vomiting **unrelated to the pregnancy** include

- [Gastroenteritis](#) (an infection of the stomach or intestines)
- Disorders of the abdomen such as [appendicitis](#), a blockage in the intestine ([intestinal obstruction](#)), or inflammation of the gallbladder ([cholecystitis](#))
- [Migraine headaches](#) or more severe brain disorders)

However, these disorders usually cause other symptoms that are more prominent, such as abdominal pain or headaches.

Evaluation of Nausea and Vomiting During Pregnancy

Doctors first try to determine whether nausea and vomiting are caused by a serious disorder. Morning sickness and hyperemesis gravidarum are diagnosed only after other causes are ruled out.

Warning signs

In pregnant women who are vomiting, the following symptoms are cause for concern:

- Vomiting that persists or that is worsening
- Abdominal pain
- Signs of [dehydration](#), such as decreased urination, decreased sweating, increased thirst, a dry mouth, a rapid heart rate, and dizziness when standing up
- Fever
- No movement of the fetus if the pregnancy is 24 or more weeks
- Confusion, weakness or numbness of one side of the body, speech or vision problems, or sluggishness

When to see a doctor

Women with warning signs should see a doctor right away.

Women without warning signs should talk to their doctor. The doctor can help them decide whether and how quickly they need to be seen based on the nature and severity of their symptoms. Women who have mild to moderate nausea and vomiting, have not lost weight, and are able to keep some liquids down may not need to see a doctor unless their symptoms worsen.

What the doctor does

Doctors ask about symptoms and the medical history and do a physical examination. What they find during the history and physical examination often suggests a cause and the tests that may need to be done (see table

[Some Causes and Features of Nausea and Vomiting During Early Pregnancy](#)

Doctors ask about the vomiting:

- When it started
- How long it lasts
- How many times a day it occurs
- Whether anything relieves or makes it worse
- How much there is
- If food and/or liquids are staying down

The doctor asks the woman if she has other symptoms, particularly abdominal pain, diarrhea, and constipation, and how her symptoms have affected her and her family—

whether she can work and care for herself and her children. The doctor also asks about vomiting in previous pregnancies, about previous abdominal surgery, and use of medications that may contribute to vomiting.

During the physical examination, doctors do a routine prenatal evaluation, including measuring the woman's vital signs and evaluating the fetus. They look for signs of serious disorders, such as blood pressure that is too low or too high, fever, or abdominal tenderness.

This information helps doctors determine whether vomiting results from the pregnancy or another, unrelated disorder. For example, vomiting is more likely to be due to pregnancy if

- Symptoms begin during the 1st trimester.
- Symptoms last or recur over several days to weeks.
- The woman does not have abdominal pain.
- There are no symptoms involving other organ systems.

Vomiting probably results from a disorder unrelated to the pregnancy if it

- Began after the 1st trimester
- Is accompanied by abdominal pain, diarrhea, or both



Some Causes and Features of Nausea and Vomiting During Early Pregnancy

Cause	Common Features	Diagnostic Approach†
Related to the pregnancy (obstetric)		
Nausea and vomiting of pregnancy (morning sickness)	Mild nausea and vomiting that comes and goes and that occurs at varying times throughout the day, primarily during the 1st trimester	A doctor's examination
<u>Hyperemesis gravidarum</u>	Frequent, persistent nausea and vomiting Inability to consume enough fluids, food, or both Usually signs of dehydration, such as decreased urination, decreased sweating, a dry mouth, increased thirst, a rapid heart rate, and dizziness when standing up Weight loss	Blood tests to check for signs of dehydration and chemical imbalances by measuring levels of electrolytes, blood urea nitrogen (BUN), creatinine, and liver enzymes Urine tests to measure ketones (produced when not enough food is consumed and the body breaks down fats for energy)
 MSD Copyright © 2026 Merck & Co., Inc., Rahway, NJ, USA and its affiliates. All rights reserved. A <u>molar pregnancy</u> (abnormal placental growth with or without a fetus due to an abnormally fertilized egg)	A uterus that is larger than expected for the weeks of pregnancy No heartbeat or movement detected in the fetus Later in pregnancy, sometimes high blood pressure, swelling of	Blood tests to measure human chorionic gonadotropin (hCG—a hormone produced by the placenta early in pregnancy) Ultrasonography of the pelvis